All sites · All lead groups · All roles · All phases · Any status · Protocol not limited · My trials sample

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Breast Cancer

A012103

Phase 3 · ALLIANCE · Open · NCT05812807

OptimICE-PCR: De-Escalation of Therapy in Early-Stage TNBC Patients Who Achieve pCR After Neoadjuvant Chemotherapy with Checkpoint Inhibitor Therapy

Entry. Clinical stage II-III TNBC with pCR after neoadjuvant chemotherapy plus pembrolizumab (at least 6 cycles) and definitive surgery. Randomize within 12 weeks of final surgery and within 14 days of consent.

Stratification. Nodal status before preoperative chemotherapy (node-positive by FNA, core biopsy, or sentinel node vs node-negative); receipt of preoperative anthracycline (yes vs no).

Randomize 1:1. Pocock-Simon dynamic allocation. Protocol treatment begins within 7 days of registration.

Arm 1: Pembrolizumab x 27 weeks Physician choice: 200 mg IV every 3 weeks, or 400 mg IV every 6 weeks x 4 doses then 200 mg x 1 dose. Subcutaneous option: 395 mg every 3 weeks or 790 mg every 6 weeks. CCTG sites may use 2 mg/kg IV (max 200 mg) every 3 weeks.

Arm 2: Observation x 27 weeks No protocol treatment. Visits at baseline, 12 weeks, and 27 weeks after registration.

Adjuvant radiation. Standard adjuvant RT is allowed on study (preferably completed before registration) and may be given concurrently with pembrolizumab. It is not protocol treatment.

Follow-up. 5 years from registration or recurrence, then annually for overall survival to 10 years.

Primary endpoint. Recurrence-free survival, non-inferiority of observation. Assumed 3-year RFS 94% with pembrolizumab; observation non-inferior if 3-year RFS is 91% or higher.

1,125 of 1,295 accrued (86.9%). open since Jun 2023. 28.2 per month on average. projected Apr 2027 at that rate

Breast Cancer

A012303

Phase 3 · ALLIANCE · Open · NCT06876714

ShortStop-HER2: Shortened Duration of Adjuvant Therapy in Patients with Early-Stage HER2+ Breast Cancer Who Achieve pCR After Neoadjuvant Chemotherapy with HER2 Blockade

Entry. cT1c-T3 (or Tx) N0-N1 HER2+ breast cancer (cT3N1 excluded) with pCR after neoadjuvant chemotherapy plus trastuzumab +/- pertuzumab. Register within 14 weeks of final surgery, after 12-24 weeks of prior trastuzumab coverage.

Stratification. Baseline stage I/IIA vs IIB; ER positive vs negative; neoadjuvant chemotherapy 4 cycles vs more than 4; planned adjuvant pertuzumab yes vs no.

Randomize 1:1. Pocock-Simon dynamic allocation. Treatment starts within 14 days of registration. Trastuzumab +/- pertuzumab (IV or SC, investigator choice, declared before randomization) every 21 days. Total duration counts HER2 blockade given before registration.

Arm 1: 12 months (standard) 51 weeks total of trastuzumab-containing therapy (17 cycles of 21 days).

Arm 2: 6 months 27 weeks total of trastuzumab-containing therapy (9 cycles of 21 days).

Dosing. Trastuzumab IV 8 mg/kg load then 6 mg/kg; pertuzumab IV 840 mg load then 420 mg; trastuzumab-hyaluronidase SC 600 mg; Phesgo SC load 1,200/600 mg then 600/600 mg. First on-study dose at least 21 days after the last pre-registration dose.

Concurrent standard therapy. Adjuvant RT per local standards (encouraged concurrently with HER2 therapy) and adjuvant endocrine therapy for HR+ disease. Neither is protocol treatment.

Follow-up. Every 6 months for 5 years or until recurrence, then annually for survival to 10 years.

Primary endpoint. Recurrence-free survival, non-inferiority of 6 vs 12 months. HR margin 1.63, assumed 3-year RFS 95% in the control arm; 1,524 patients, 113 RFS events.

269 of 1,524 accrued (17.7%). open since Sep 2025. 21 per month on average. projected Oct 2031 at that rate

NRG-BR009 (OFSET)

Phase 3 · NRG · Open · NCT05879926

A Phase III Adjuvant Trial Evaluating the Addition of Adjuvant Chemotherapy to Ovarian Function Suppression Plus Endocrine Therapy in Premenopausal Patients with pN0-1, ER-Positive/ HER2-Negative Breast Cancer and an Oncotype Recurrence Score </= 25 (OFSET)

Population. Premenopausal, resected ER-positive/HER2-negative breast cancer: pN0 with Oncotype RS 21-25, or RS 16-20 with high clinical risk (low grade and >3 cm, intermediate grade and >2 cm, or high grade and >1 cm); or pN1 with RS 0-25.

Step 1: Pre-entry. Registration. Patients qualifying by MammaPrint Low Risk/MP1 need central Oncotype DX RS before randomization; MammaPrint High Risk/MP2 are ineligible. Intended RT plan declared before pre-entry.

Stratification. Nodal/RS status (pN0 RS 16-25 vs pN1 RS 0-15 vs pN1 RS 16-25), intent to receive a CDK4/6 inhibitor (yes vs no), age (18-39 vs 40 or older).

Step 2: Randomize 1:1. Stratified permuted blocks.

Arm 1 Ovarian function suppression (GnRH agonist of investigator's choice, or bilateral oophorectomy) plus aromatase inhibitor for 5 years.

Arm 2 Adjuvant chemotherapy of investigator's choice (NCCN- or ESMO-listed regimen), then ovarian function suppression plus aromatase inhibitor for 5 years.

Radiotherapy (both arms). Whole breast irradiation required after lumpectomy (partial breast irradiation not permitted); post-mastectomy and regional nodal RT per protocol guidelines by nodal status. Allowed WBI regimens: 50 Gy/25, 42.56 Gy/16, 40 Gy/15, or 26 Gy/5, with optional sequential or concomitant boost. OFS plus AI preferably started before and given with RT, or within 4 weeks after RT.

Other therapy. CDK4/6 inhibitor (abemaciclib or other FDA-approved) and bone anti-resorptive therapy allowed per standard use.

Primary endpoint. Invasive breast cancer-free survival.

737 of 3,960 accrued (18.6%). open since Oct 2023. 20.6 per month on average. projected Oct 2039 at that rate

Breast Cancer

S1501

Phase 3 · SWOG · Open · NCT03418961

S1501, Prospective Observational Cohort Study of Patients with Metastatic HER-2+ Breast Cancer at Risk of Cardiac Toxicity

Site requirement. Site must first be approved as an S1501 Validated ECHO Lab by the central ECHO Core Lab.

Step 1: Registration. Metastatic HER2-positive breast cancer on or starting trastuzumab-based therapy, at increased cardiac risk. Baseline ECHO (within 28 days) submitted for central read.

Step 2: Registration. Central LVEF ≥ 50% confirmed; register within 21 days of ECHO submission. Cohort assigned by current beta blocker, ARB, or ACE inhibitor use.

Arm 3: Observation (only arm open). Patients already taking a beta blocker, ARB, or ACE inhibitor continue it; no study drug. Standard-of-care ECHOs (expected every 12 weeks) submitted centrally, with blood specimens, for 108 weeks or until cardiac dysfunction.

Closed randomized cohort. Patients not on these agents were randomized (stratified by prior anthracycline and central LVEF 50-54% vs ≥ 55%) to Arm 1, carvedilol 6.25 mg PO twice daily for 108 weeks, or Arm 2, no prophylaxis. Arms 1 and 2 are permanently closed to accrual as of Revision 7.

Endpoint assessment. Cardiac dysfunction by central ECHO Core Lab read and predefined cardiac events; HER2 therapy continuation is at physician discretion.

Primary objective. 2-year cumulative incidence of cardiac dysfunction and/or predefined cardiac events in patients on trastuzumab-based therapy plus a beta blocker, ARB, or ACE inhibitor, and a risk prediction model from clinical factors and biomarkers.

402 of 491 accrued (81.9%). open since Nov 2017. 3.8 per month on average. projected Oct 2028 at that rate

Breast Cancer

S2212

Phase 3 · SWOG · Open · NCT05929768

Shorter Anthracycline-Free Chemo Immunotherapy Adapted to Pathological Response in Early Triple Negative Breast Cancer (SCARLET), A Randomized Phase III Study

Entry. Early triple-negative breast cancer, cT2-4 N0, cT1-3 N1-2, or TX N1-2, M0 (no T4/N+, N3, or inflammatory)

Stratification. Nodal status negative vs positive (TIL status used as an analysis stratum)

Randomize 1:1. Neoadjuvant chemo-immunotherapy, starting within 21 days

Arm 1: Carboplatin/paclitaxel/pembrolizumab then AC/pembrolizumab Cycles 1-4 (21 days): paclitaxel 80 mg/m2 days 1, 8, 15 plus carboplatin AUC 1.5 weekly or AUC 5 day 1, plus pembrolizumab 200 mg every 3 weeks. Cycles 5-8: doxorubicin 60 mg/m2 + cyclophosphamide 600 mg/m2 every 21 or 14 days, pembrolizumab every 3 weeks (8 neoadjuvant doses)

Arm 2: Carboplatin/docetaxel/pembrolizumab (anthracycline-free) Docetaxel 75 mg/m2 + carboplatin AUC 6 + pembrolizumab 200 mg day 1 every 21 days x 6 cycles with required growth factor

Surgery. Definitive surgery recommended 3-10 weeks after last neoadjuvant dose

Adjuvant. Pembrolizumab recommended for all (total 55 weeks). pCR: no further chemotherapy. Residual disease: Arm 1 capecitabine at physician discretion; Arm 2 AC x 4 with pembrolizumab at physician discretion, with or without capecitabine; olaparib allowed for germline BRCA. Radiation per standard guidelines

Follow-up. 5 years from registration

Primary endpoint. Breast cancer event-free survival (non-inferiority of Arm 2)

1,733 of 2,400 accrued (72.2%). open since Sep 2023. 47 per month on average. projected Dec 2027 at that rate

Female Reproductive System Cancer

NRG-CC008

Phase Other · NRG · Open · NCT04251052

A Non-Randomized Prospective Clinical Trial Comparing the Non-Inferiority of Salpingectomy to Salpingo-oophorectomy to Reduce the Risk of Ovarian Cancer Among BRCA1 Carriers [SOROCk]

Population. Age 35-50 with a germline pathogenic or likely pathogenic BRCA1 mutation, at least one ovary and fallopian tube in situ. Non-randomized: surgery chosen by the patient after standardized counseling that RRSO is standard of care.

Age groups. Age 35-40: 1:1 allocation BLS:BSO. Age 41-50: 1:2 allocation BLS:BSO.

BLS cohort. Bilateral salpingectomy with delayed oophorectomy (concurrent hysterectomy allowed), within 180 days of registration. Annual re-counseling that oophorectomy by age 40 is recommended; crossover to completion oophorectomy allowed any time, analyzed intention-to-treat.

BSO cohort. Risk-reducing bilateral salpingo-oophorectomy (concurrent hysterectomy allowed), within 180 days of registration. HRT encouraged after BSO if no contraindication.

Pathology. SEE-FIM processing of all specimens; STIC or invasive cancer sent for central review.

Primary endpoint. Time to incident high-grade serous ovarian, primary peritoneal, or fallopian tube carcinoma (non-inferiority of BLS to BSO).

1,187 of 1,956 accrued (60.7%). open since Sep 2020. 16.2 per month on average. projected Sep 2030 at that rate

Female Reproductive System Cancer

NRG-GY035

Phase 3 · NRG · Open · NCT07198074

A Randomized Phase III Trial of Carboplatin, Paclitaxel, Pembrolizumab Versus Carboplatin, Paclitaxel, Bevacizumab Versus Carboplatin, Paclitaxel, Pembrolizumab, Bevacizumab in the Treatment of pMMR, TP53 Mutated Advanced or Recurrent Endometrial Cancer

Population. pMMR, TP53-mutated endometrial cancer: stage III or IVA with measurable disease, stage IVB with or without measurable disease, or recurrent with or without measurable disease.

Stratification. Disease status (newly diagnosed vs recurrent), prior adjuvant chemotherapy (yes vs no).

Randomize 1:1:1. Permuted blocks. Combination phase: paclitaxel 175 mg/m2 and carboplatin AUC 5 every 3 weeks for 6-10 cycles (beyond 6 only for SD or PR with residual measurable disease). Treatment starts within 14 days of registration.

Arm 1 (reference) Paclitaxel, carboplatin, pembrolizumab 200 mg every 3 weeks, then pembrolizumab 400 mg every 6 weeks for 14 additional cycles.

Arm 2 Paclitaxel, carboplatin, bevacizumab (or anti-VEGF biosimilar) 15 mg/kg every 3 weeks, then bevacizumab 15 mg/kg every 3 weeks for 28 additional doses.

Arm 3 Paclitaxel, carboplatin, pembrolizumab 200 mg, and bevacizumab 15 mg/kg every 3 weeks, then pembrolizumab 400 mg every 6 weeks for 14 cycles plus bevacizumab 15 mg/kg every 3 weeks for 28 doses.

Imaging. Every 9 weeks for the first year, then every 12 weeks.

Primary endpoint. Progression-free survival, each experimental arm vs Arm 1; interim futility and efficacy analysis at 60% information. PFS not reported until accrual completes.

53 of 255 accrued (20.8%). open since Jan 2026. 6.3 per month on average. projected Jun 2029 at that rate

Female Reproductive System Cancer

NRG-GY036

Phase 3 · NRG · Open · NCT06580314

A Phase III Trial of One vs. Two Years of Maintenance Olaparib, with or Without Bevacizumab, in Patients with BRCA1/ 2 Mutated or Homologous Recombination Deficient (HRD+) Ovarian Cancer Following Response to First Line Platinum-Based Chemotherapy

Population. Newly diagnosed FIGO stage III-IV ovarian, fallopian tube, or primary peritoneal cancer with a pathogenic BRCA1/2 alteration (germline or somatic) or BRCA wild-type HRD-positive tumor, in PR or CR after first-line platinum-based chemotherapy.

Step 1: Registration. Randomized 3-12 weeks after the last platinum dose. Physician documents the bevacizumab decision before randomization.

Stratification. BRCA deleterious alteration vs BRCA wild-type HRD-positive; bevacizumab (physician's choice) yes vs no; response to platinum (PR vs CR).

Randomize 1:1. 21-day cycles; treatment starts within 14 days of registration.

Arm 1 (reference) Olaparib 300 mg PO twice daily for 2 years, with or without bevacizumab 15 mg/kg IV every 21 days (bevacizumab stops at 1 year).

Arm 2 Olaparib 300 mg PO twice daily for 1 year, with or without bevacizumab 15 mg/kg IV every 21 days for up to 1 year.

Primary endpoint. Investigator-assessed PFS by RECIST 1.1 (non-inferiority of 1 year), analyzed as PFS360: modified ITT population of patients on study at least 360 days after randomization, with time at risk starting at day 360.

366 of 880 accrued (41.6%). open since Mar 2025. 19.3 per month on average. projected Dec 2028 at that rate

Female Reproductive System Cancer

NRG-GY037

Phase 3 · NRG · Open · NCT07061977

A Phase III Study of Induction Pembrolizumab and Chemotherapy Followed by Chemoradiation and Pembrolizumab vs Chemoradiation and Pembrolizumab Both Followed by Pembrolizumab for High Risk Locally Advanced Cervical Cancer

Population. Newly diagnosed squamous cell carcinoma, adenocarcinoma, or adenosquamous carcinoma of the cervix, FIGO 2018 stage IIIA, IIIB, IIIC1, IIIC2, or IVA.

Stratification. FIGO stage III versus IVA, and presence or absence of para-aortic nodes (N2). Randomize 1:1.

Arm 1. Concurrent chemoradiation: weekly cisplatin 40 mg/m2 for 5 weeks, external-beam radiotherapy, brachytherapy, and pembrolizumab 200 mg every 3 weeks for 5 cycles, then pembrolizumab 400 mg every 6 weeks for 14 maintenance cycles.

Arm 2. Induction weekly carboplatin AUC 2 and paclitaxel 80 mg/m2 for 6 weeks, plus pembrolizumab 200 mg every 3 weeks for 2 cycles, then the same chemoradiation and pembrolizumab, then pembrolizumab 400 mg every 6 weeks for 15 maintenance cycles.

102 of 336 accrued (30.4%). open since Nov 2025. 9.1 per month on average. projected Nov 2028 at that rate

Gastrointestinal Cancer

EA2192

Phase 2 · ECOG-ACRIN · Open · NCT04858334

APOLLO: A Randomized Phase II Double-Blind Study of Olaparib Versus Placebo Following Curative Intent Therapy in Patients with Resected Pancreatic Cancer and a Pathogenic BRCA1, BRCA2 or PALB2 Mutation

No schema is on file for this protocol brief.

119 of 152 accrued (78.3%). open since Jun 2021. 1.9 per month on average. projected Mar 2028 at that rate

NRG-CC005 (FORTE)

Phase 3 · NRG · Open · NCT05080673

FORTE (Five or Ten Year Colonoscopy for 1-2 Non-Advanced Adenomatous Polyps)

Population. Age 45 or older and under 70 at qualifying colonoscopy, with a first diagnosis of 1-2 non-advanced adenomas at a complete qualifying colonoscopy within the previous 4 years.

Stratification. Age at qualifying colonoscopy (45-55 vs 56 to under 70), sex, time from qualifying colonoscopy to randomization (under 2 years vs 2-4 years).

Randomize 1:1. Dynamic minimization with biased coin.

Arm 1 Surveillance colonoscopy at 5 years and 10 years after the qualifying colonoscopy (each +/- 1 year).

Arm 2 Surveillance colonoscopy at 10 years after the qualifying colonoscopy (+/- 1 year).

Follow-up. Annual contact for disease and vital status; long-term follow-up up to 30 years from randomization.

Primary endpoint. Colorectal cancer incidence in years 4 through 11 (non-inferiority of the 10-year-only schedule).

5,282 of 9,500 accrued (55.6%). open since Feb 2022. 94.3 per month on average. projected Jul 2030 at that rate

Gastrointestinal Cancer

NRG-GI008 (CIRCULATE-US)

Phase 2/3 · NRG · Open · NCT05174169

Colon Adjuvant Chemotherapy Based on Evaluation of Residual Disease (CIRCULATE-NORTH AMERICA)

Population. R0-resected stage IIB, IIC, or III colon adenocarcinoma, pMMR/MSS.

Step 1: Registration. Central Signatera ctDNA testing of postoperative blood (recommended 14 or more days after surgery) and resected tumor. One optional cycle of 5-FU (2 weeks) or capecitabine (3 weeks) with or without oxaliplatin may be given after consent, starting 4 or more weeks after surgery and completed before randomization; ctDNA blood must be drawn before it.

Cohort A (ctDNA negative): Step 2 Randomize 1:1. Stratified by stage (II/IIIA vs IIIB vs IIIC) and intended fluoropyrimidine (5-FU vs capecitabine).

Arm 1 Immediate adjuvant chemotherapy: mFOLFOX6 for 3-6 months (6-12 cycles) or CAPOX for 3 months (4 cycles).

Arm 2 Serial ctDNA surveillance every 3 months through year 1, then every 6 months through year 3. Conversion to ctDNA positive allows transfer to Cohort B with a second randomization to Arm 3 or 4.

Cohort B (ctDNA positive): Step 2 Randomize 1:1. Stratified by intended fluoropyrimidine (5-FU vs capecitabine) and initial post-op ctDNA status (positive vs negative, the latter being Arm 2 converters).

Arm 3 mFOLFOX6 (12 cycles) or CAPOX (8 cycles) for 6 months.

Arm 4 mFOLFIRINOX for 6 months (12 cycles): oxaliplatin 85 mg/m2, leucovorin 400 mg/m2, irinotecan 150 mg/m2, 5-FU 2400 mg/m2 over 46-48 hours every 2 weeks, no 5-FU bolus.

Timing. Chemotherapy within 4 weeks of randomization and preferably no later than 12 weeks after surgery.

Primary endpoints. Cohort A: phase II time to ctDNA positivity (or recurrence), phase III DFS. Cohort B: DFS (phase II and III).

1,469 of 1,912 accrued (76.8%). open since Jul 2022. 28.8 per month on average. projected Jan 2028 at that rate

Genitourinary Cancers

A032201 (STRIKE)

Phase 3 · ALLIANCE · Open · NCT06661720

Short TeRm Intensified Pembrolizumab (KEytruda) and Tivozanib for High-Risk Renal Cell Carcinoma - STRIKE

Registration. Resected intermediate-high-risk, high-risk, or M1 NED clear cell RCC, more than 4 and up to 16 weeks after surgery, with no evidence of disease. Treatment starts within 14 days.

Stratification. Pathologic stage: T2 or T3 N0 vs TxN1 or T4N0 vs M1 NED.

Randomize 1:1. 12-week cycles, 4 cycles (48 weeks) of pembrolizumab.

Arm 1: Pembrolizumab 400 mg IV every 6 weeks for up to 8 doses (preferred), or 200 mg IV every 3 weeks for up to 16 doses, per institutional practice.

Arm 2: Pembrolizumab plus tivozanib Pembrolizumab 400 mg IV every 6 weeks for up to 8 doses, plus tivozanib 1.34 mg PO daily Days 1-21 of each 28-day period for up to 24 weeks (cycles 1-2).

Follow-up. Exams and scans to 5 years from registration, then survival annually to 10 years.

Primary endpoint. Investigator-assessed disease-free survival. Assumed 24-month DFS 77% with pembrolizumab, target HR 0.67 (84%); target accrual 1,040.

400 of 1,040 accrued (38.5%). open since Mar 2025. 21.2 per month on average. projected Apr 2029 at that rate

NRG-GU015 (ARCHER)

Phase 3 · NRG · Open · NCT07097142

The Phase III Adaptive Radiation and Chemotherapy for Muscle Invasive Bladder Cancer Trial (ARCHER)

Disease. cT2-cT3N0 muscle-invasive urothelial carcinoma of the bladder.

Stratify. Clinical stage T2 vs T3. Planned radiosensitizer: cisplatin, gemcitabine, or mitomycin-C/5-FU. Neoadjuvant therapy yes vs no.

Randomize 1:1. Protocol treatment starts within 28 days after randomization. Arm 2 must use plan-of-the-day or online adaptive radiotherapy.

Arm 1 4 weeks of radiosensitizing chemotherapy plus hypofractionated radiotherapy, 55 Gy in 20 fractions.

Arm 2 4 weeks of radiosensitizing chemotherapy plus SBRT, 32.5 Gy in 5 fractions.

Primary question. Non-inferiority of ultra-hypofractionation versus hypofractionation for 3-year bladder-intact event-free survival. The margin is 10% (50% to 40%), corresponding to a hazard ratio below 1.32.

97 of 486 accrued (20%). open since Oct 2025. 8 per month on average. projected Oct 2030 at that rate

Genitourinary Cancers

S2427 (BRIGHT)

Phase 2 · SWOG · Open · NCT07061964

Single Arm Phase II Study of Bladder Preservation with Immunoradiotherapy After a Clinically Meaningful Response to Neoadjuvant Therapy in Patients with Muscle Invasive Bladder Cancer (BRIGHT)

Muscle-invasive disease. Histologic cT2-T4aN0 urothelial carcinoma of the bladder.

Neoadjuvant therapy. Physician's choice, limited to the FDA-approved regimens in the eligibility criteria.

Response assessment. cT0 or cT1 on a repeat TURBT with systematic biopsies. That TURBT is within 90 days before registration.

Registration. Within 90 days after the last neoadjuvant dose.

Treatment. Bladder radiotherapy plus pembrolizumab for 1 year. Central radiation-plan approval is required before treatment starts. Radiotherapy is whole bladder only, 55 Gy in 20 fractions of 2.75 Gy, with no pelvic nodes and no tumor boost.

On-study assessment. Cystoscopy, cytology, and imaging.

Endpoint. 3-year bladder-intact event-free survival. Follow-up continues for 5 years after registration.

56 of 111 accrued (50.5%). open since Nov 2025. 5.3 per month on average. projected Aug 2027 at that rate

Head and Neck Cancer

A222301 (HOPE)

Phase 3 · ALLIANCE · Open · NCT06992427

High-Dose Prophylactic Gabapentin (HOPE) to Prevent Opioid Use for Oral Mucositis Pain During Head and Neck Chemoradiotherapy: A Phase III Clinical Trial

Arm A. Cisplatin-based chemoradiation, placebo, and the institution's standard treatment for mucositis.

Arm B. Cisplatin-based chemoradiation, prophylactic gabapentin titrated to 1200 mg three times a day, and the institution's standard treatment for mucositis.

Duration. About 10 weeks: a 12-day ramp up that may start any time from randomization through day 8 of chemoradiation, 7 weeks during radiotherapy, and a 12-day ramp down.

26 of 228 accrued (11.4%). open since Jun 2025. 1.6 per month on average. projected Apr 2037 at that rate

EAA173

Phase 3 · ECOG-ACRIN · Open · NCT03937635

Daratumumab to Enhance Therapeutic Effectiveness of Revlimid in Smoldering Myeloma (DETER-SMM)

Step 0: Pre-registration. Potential candidates; pre-study specimens per consent.

Step 1: Randomization. High-risk smoldering multiple myeloma diagnosed within the past 12 months. Accrual goal 288. Cycle = 28 days.

Stratification. Age at high-risk SMM diagnosis (< 65 vs >= 65 years).

Randomize (equal allocation). Treatment for 24 cycles.

Arm A: DRd Daratumumab-hyaluronidase 1800 mg/30,000 units SC (or daratumumab 16 mg/kg IV if SC not tolerated) days 1, 8, 15, 22 cycles 1-2, days 1 and 15 cycles 3-6, day 1 cycles 7-24; lenalidomide 25 mg PO days 1-21 cycles 1-24 (10 mg if CrCl 30-59 ml/min); dexamethasone 40 mg PO days 1, 8, 15, 22 cycles 1-6, then 20 mg cycles 7-12.

Arm B: Rd Lenalidomide 25 mg PO days 1-21 cycles 1-24 (10 mg if CrCl 30-59 ml/min); dexamethasone 40 mg PO days 1, 8, 15, 22 cycles 1-6, then 20 mg cycles 7-12.

Stem cell collection. Peripheral blood stem cell collection suggested between cycles 4-6; therapy may be interrupted up to 6 weeks. Not required.

Observation. Followed for response until progression and for survival for 15 years from randomization.

Primary endpoint. Overall survival.

267 of 288 accrued (92.7%). open since Sep 2019. 3.2 per month on average. projected May 2027 at that rate

Lung, Mediastinal, and Pleural Cancer

A081801

Phase 3 · ALLIANCE · Open · NCT04267848

Integration of Immunotherapy Into Adjuvant Therapy for Resected NSCLC: ALCHEMIST Chemo-IO (ACCIO)

Eligibility. Completely resected (R0) stage IIA, IIB, IIIA, or IIIB (T3-4N2) NSCLC per AJCC 8th edition, EGFR and ALK negative locally, registered 30-77 days after surgery. 1 cycle = 21 days.

Stratification. Histology (squamous vs non-squamous), local PD-L1 TPS (50% or higher vs below 50% or not evaluable), smoking status (never vs former/current), pathologic stage (IIA/IIB vs IIIA/IIIB T3-4N2).

Randomize. Since Update #07, randomization is to Arms B and C (the former Arm A, platinum doublet then observation, is closed). Platinum doublet is physician's choice: cisplatin/pemetrexed or carboplatin/pemetrexed for non-squamous, cisplatin/gemcitabine or carboplatin/paclitaxel for squamous.

Arm B (sequential) Platinum doublet x 4 cycles, +/- PORT, then pembrolizumab 400 mg IV every 6 weeks x 8 doses (16 cycles)

Arm C (concurrent) Platinum doublet x 4 cycles with pembrolizumab 400 mg on day 1 of cycles 1 and 3, +/- PORT, then pembrolizumab 400 mg every 6 weeks x 6 doses (12 cycles), 16 total cycles of pembrolizumab

PORT (optional). At treating physician discretion after the 4 chemotherapy cycles; recommended for occult pN2/N3, R1 margins, or extranodal extension, not for tumor size or pleural involvement alone. 50-54 Gy in 2 Gy (25-27 fractions) or 1.8 Gy (28-30 fractions). Starts 2-6 weeks after the last chemotherapy infusion and before continuation pembrolizumab, with no concurrent systemic therapy; pembrolizumab resumes 5-21 days after PORT.

Follow-up. Up to 10 years or until death. Primary endpoint DFS.

919 of 1,210 accrued (76%). open since Jun 2020. 12.1 per month on average. projected Oct 2028 at that rate

LUNGMAP

Phase Other · SWOG · Open · NCT03851445

A Master Protocol to Evaluate Biomarker-Driven Therapies and Immunotherapies in Previously-Treated Non-Small Cell Lung Cancer (Lung-MAP Screening Study)

LUNGMAP registration. Three entry routes: pre-screening before progression on current therapy (tissue only), screening at progression (tissue or prior NGS results), or treatment-naive NSCLC with a designated targetable alteration (prior NGS results only, when such sub-studies are open).

Biomarker profiling. Tissue sent to Foundation Medicine within 5 days of registration for broad CLIA NGS profiling, or upload of a previously completed approved NGS report via the Genomic Alterations Form.

Sub-study assignment. Biomarker-matched sub-study if a qualifying alteration is found (lowest prevalence or randomized assignment if several match); otherwise a non-match sub-study if one is accruing. Assignment within about 16 days of tissue submission, within 1 day for prior test results; pre-screened patients receive assignment after the Notice of Progression is submitted.

Sub-study registration. Separate registration to the assigned sub-study, which has its own eligibility and treatment. After progression on a sub-study, a new assignment may be requested.

Follow-up. Until sub-study registration, death, or 3 years after LUNGMAP registration; after sub-study registration, until death or 3 years.

Primary objective. Screening protocol: determine eligibility for the biomarker-driven and non-match Lung-MAP sub-studies.

open since Feb 2019

Lung, Mediastinal, and Pleural Cancer

S1800E

Phase 2/3 · SWOG · Open · NCT06616584

A Randomized Phase II/ III Study of Docetaxel and Ramucirumab with or Without Cemiplimab (REGN2810) for Participants Previously Treated with Platinum-Based Chemotherapy and Immunotherapy for Stage IV or Recurrent Non-Small Cell Lung Cancer (Lung-MAP Non-Matched Sub-Study)

LUNGMAP registration. Previously treated stage IV or recurrent NSCLC screened on the Lung-MAP master protocol; not eligible for a biomarker-matched sub-study and assigned to S1800E as a non-match sub-study.

Stratify. Histology (squamous or mixed vs non-squamous); Zubrod performance status (0 vs 1).

Randomize. 21-day cycles starting within 7 days of randomization, until progression, unacceptable toxicity, or treatment delay > 84 days. Randomization ratio not stated in the protocol text.

Arm A (control) Ramucirumab 10 mg/kg IV Day 1 + docetaxel 75 mg/m2 IV Day 1 (60 mg/m2 allowed) every 21 days, with dexamethasone premedication.

Arm B Ramucirumab + docetaxel as in Arm A + cemiplimab 350 mg IV Day 1 every 21 days. Cycle 1 safety run-in with weekly toxicity checks.

Primary endpoint. Overall survival (phase III), with a phase II interim futility analysis on PFS at 99 events. Accrual goal 378.

Follow-up. Until death or 3 years after randomization.

144 of 378 accrued (38.1%). open since May 2025. 8.6 per month on average. projected Jan 2029 at that rate

Male Reproductive System Cancer; Genitourinary Cancers

NRG-GU013 (High Five)

Phase 3 · NRG · Open · NCT05946213

The Phase III 'High Five Trial' Five Fraction Radiation For High-Risk Prostate Cancer

Disease. Pathologically proven adenocarcinoma of the prostate, meeting the high-risk definition in eligibility.

Stratify. Pelvic nodal radiotherapy yes vs no. ADT duration under 18 months vs over 18 months. Second-generation antiandrogen yes vs no. Intraprostatic microboost yes vs no.

Randomize 1:1. ADT of 6 to 36 months is required. Intended duration, type, and use of a second-generation antiandrogen are declared at registration.

Arm 1 Ultrahypofractionated SBRT in 5 fractions.

Arm 2 Conventional or moderate hypofractionation in 20 to 45 fractions.

Primary endpoint. Metastasis-free survival on conventional imaging.

1,201 of 1,500 accrued (80.1%). open since Dec 2023. 35.4 per month on average. projected Jun 2027 at that rate

Miscellaneous and Metastatic Cancer

A151804

Phase Other · ALLIANCE · Open · NCT04242095

Establishment of a National Biorepository to Advance Studies of Immune-Related Adverse Events

Step 0: Pre-registration (optional, before IO therapy). Patients planning to start an immune checkpoint or other IO regimen (Appendix I), IO-naive. Baseline blood (mandatory), stool (optional), and archival tumor tissue collected after consent and before IO treatment. Repeat blood/stool and data at 1 month (± 14 days), with chart review at 6 and 12 months (± 1 month) after pre-registration.

Step 1: Registration at irAE. Within 96 hours of research team awareness, clinical diagnosis, or test confirmation of a qualifying irAE (grade 3-4 irAE, grade 2 dermatologic/endocrine/rheumatologic irAE, rare infection, or hyperprogression). Patients may pre-register and register concurrently.

Collection. Blood within 24 hours of registration, stool (optional), diagnostic irAE biopsy tissue within 7 days, imaging (hyperprogression, pneumonitis, imaging-diagnosed irAEs) and photos (dermatologic irAEs). Repeat collection at 1 month (± 14 days) after registration and at any irAE recurrence; data every 3 months for 1 year.

Primary objective. Establish a national biorepository of biospecimens and clinical data from IO-treated patients with serious irAEs (myocarditis, colitis, hepatitis, nephritis, myositis, pneumonitis, meningitis/encephalitis, dermatitis, endocrinopathies, neuropathy, other rheumatologic, cytopenias, pancreatitis, rare infections, hyperprogression).

511 of 360 accrued (141.9%). open since Mar 2020. 6.5 per month on average

Miscellaneous and Metastatic Cancer

A211901

Phase 3 · ALLIANCE · Open · NCT05008848

Reaching Rural Cancer Survivors Who Smoke Using Text-Based Cessation Interventions

Registration. Rural cancer survivors (diagnosis within 10 years) who smoke ≥ 5 cigarettes/day and want to quit.

Stratification. Baseline cigarettes per day (≥ 20 vs < 20); age (≥ 65 vs < 65); sex. Pocock-Simon dynamic allocation.

Randomize 1:1.

Intervention 8-week text-based Scheduled Gradual Reduction (Mosio system) with target quit date 8 weeks after enrollment, plus NCI Smokefree.TXT from 2 weeks before to 4 weeks after the quit date (12 weeks of intervention).

Control NCI Clearing the Air cessation booklet, with a quit date set within 8 weeks.

Assessments. Questionnaires at baseline, 30 days, and 6 months after quit date; urine cotinine/anabasine at 6 months for those reporting abstinence.

Primary endpoint. Biochemically validated smoking abstinence 6 months after quit date (urine cotinine < 50 ng/mL, or anabasine < 2 ng/mL for those vaping or using NRT).

373 of 429 accrued (86.9%). open since Apr 2022. 6.9 per month on average. projected Jun 2027 at that rate